JanGyan — Learn Without Limits

Category 4: Health & Emergency Awareness

Module 3 - Medical Emergencies

Module Overview

A medical emergency can happen to anyone, anywhere — at home, at work, on the road, or in a crowded market. This handbook is Module 3 of JanGyan's Health & Emergency Awareness category, and it teaches every citizen how to recognise the most serious medical emergencies and respond safely in the critical minutes before an ambulance or doctor arrives.

By the end of this handbook, you will be able to recognise the warning signs of a heart attack, stroke, and cardiac arrest, perform basic hands-only CPR, respond to fainting, seizures, diabetic emergencies, severe allergic reactions, and poisoning, and know exactly how to raise the alarm effectively. This module is designed as a complete, self-sufficient guide.

Learning Objectives

After completing this module, you will be able to:

- Recognise the warning signs of a heart attack, stroke, and cardiac arrest.
- Explain the "Chain of Survival" and why every link matters.
- Perform hands-only CPR safely and confidently.
- Respond appropriately to fainting and seizures.
- Help someone experiencing a diabetic emergency or severe allergic reaction.
- Respond safely to a suspected poisoning.
- Make an effective emergency call and guide responders to the scene.
- Separate common medical emergency myths from medically accurate facts.

Why This Topic Matters

Important

In India, a person suffers a stroke roughly every 20 seconds, and cardiac arrest survival remains below 10% — largely because bystander CPR is given in only about 1–10% of cases, far below the global target of over 60%. Learning what to do in the first few minutes of a medical emergency can genuinely change these numbers, one bystander at a time.

Doctors and paramedics cannot be everywhere at once. In the moments after a heart attack, stroke, or cardiac arrest, it is almost always a family member, a colleague, or a stranger nearby who is first on the scene. What that person does — or doesn't do — in those first few minutes often matters more than anything a hospital can do afterward.

This is why medical emergency awareness is not just "good to know" — it is a genuine public health need in India today, and one that Indian law actively encourages through Good Samaritan protections.

Introduction

A medical emergency is any sudden health event that threatens a person's life or long-term health if not treated quickly. Unlike the everyday injuries covered in JanGyan's First Aid module, the emergencies in this handbook — heart attack, stroke, cardiac arrest, severe allergic reactions, and poisoning — are all conditions where minutes genuinely determine outcomes.

Definition

A **Medical Emergency** is a sudden illness or condition that poses an immediate risk to a person's life, requiring urgent professional medical attention.

You are not expected to diagnose exactly what is wrong. Your job as a bystander is simpler and more achievable: recognise that something is seriously wrong, call for professional help immediately, and provide safe, basic support until that help arrives.

Remember

When in doubt, call for help. Emergency dispatchers would always rather receive an unnecessary call than have someone hesitate during a genuine emergency.

Fundamental Concepts

1. Recognising a Medical Emergency

Certain signs almost always mean professional help is needed immediately:

- Unconsciousness or not responding normally to voice or touch
- Absent, abnormal, or gasping breathing
- Severe, uncontrolled bleeding
- Sudden chest pain, pressure, or tightness
- Sudden weakness, confusion, or slurred speech
- A seizure or convulsion
- Signs of a severe allergic reaction (facial swelling, breathing difficulty)

2. The Chain of Survival

Resuscitation science worldwide is built around a simple idea: survival depends on a chain of actions, and if any link is missing or delayed, the chances of survival drop sharply.

Link	What It Means
1. Early Recognition & Call for Help	Spotting the emergency quickly and calling 112/108 without delay
2. Early CPR	Starting hands-only CPR immediately if the person is unresponsive and not breathing normally
3. Early Defibrillation	Using an AED (Automated External Defibrillator) as soon as one is available
4. Early Advanced Care	Paramedics and hospital teams taking over with advanced treatment

5. Post-Arrest Care	Specialised hospital care to support full recovery

Warning

Bystanders are only ever responsible for the first two links — recognising the emergency and starting CPR. You are never expected to do more than this.

Complete Detailed Explanation

Each condition below follows the same pattern: how to recognise it, and exactly what to do while waiting for professional help.

3. Heart Attack

A heart attack happens when blood flow to part of the heart muscle is suddenly blocked, usually by a clot.

- **Recognise:** Chest pain, pressure, or tightness (often spreading to the arm, jaw, neck, or back), shortness of breath, cold sweat, nausea, and light-headedness. Women may experience more subtle signs such as unusual fatigue, nausea, or back pain without obvious chest pain.
- **Respond:** Call emergency services immediately. Help the person sit down and rest in a comfortable position. Loosen tight clothing. Stay calm and reassure them. If they have their own prescribed heart medication, help them take it as directed.

Warning

Some emergency dispatchers may advise a conscious person to chew a small aspirin if they are not allergic to it — always follow the specific instructions given by the emergency operator rather than deciding this yourself. Be ready to begin CPR if the person becomes unresponsive and stops breathing normally.

4. Stroke

A stroke happens when blood flow to part of the brain is blocked or a blood vessel bursts, and brain cells begin to die within minutes.

F.A.S.T. Check	What To Look For
F — Face	Ask the person to smile. Does one side droop?
A — Arms	Ask them to raise both arms. Does one drift downward?
S — Speech	Ask them to repeat a simple phrase. Is speech slurred or strange?
T — Time	If any sign is present, call emergency services immediately and note the exact time symptoms started.

Emergency

Clot-dissolving treatment for stroke is only effective within a narrow window (commonly up to 4.5 hours from symptom onset), yet most stroke patients in India reach hospital far too late. Never give food, water, or medication by mouth — swallowing may be impaired.

5. Cardiac Arrest

Cardiac arrest is different from a heart attack — it happens when the heart suddenly stops pumping effectively, and the person becomes unresponsive and stops breathing normally, or only gasps.

- **Recognise:** The person does not respond to voice or touch, and is not breathing normally.
- **Respond:** Call emergency services immediately (or direct a bystander to), and begin hands-only CPR right away. Every minute without CPR or defibrillation sharply reduces the chance of survival.

> **Indian Context**
> Studies cited by the Cardiological Society of India estimate bystander CPR is given in only about 1.3%–9.8% of cardiac arrests in India, compared to a global target of over 60%. Fear of doing something wrong is one of the biggest reasons people hesitate — hands-only CPR was designed specifically to make this easier for untrained bystanders.

6. CPR Basics (Hands-Only CPR)

The Indian Resuscitation Council recommends hands-only CPR (chest compressions without rescue breaths) for untrained or lay bystanders, as it is simpler to perform correctly and just as effective in the first minutes of adult cardiac arrest.

1. Confirm the person is unresponsive and not breathing normally.
2. Call for emergency help, or ask someone to do so while you begin.
3. Kneel beside the person and place the heel of one hand on the centre of the chest, with the other hand on top.
4. Push hard and fast — about 100 to 120 compressions per minute, allowing the chest to fully rise between pushes.
5. Continue without stopping until the person shows signs of life, an AED arrives, or professional help takes over.

> **Tip**
> If an AED (Automated External Defibrillator) is available, switch it on and follow its voice instructions — it is designed to guide even a completely untrained person safely through its use.

7. Fainting

- **Recognise:** Dizziness, paleness, sweating, and blurred vision just before a brief loss of consciousness.
- **Respond:** Help the person lie down and gently raise their legs, or sit and lower their head between the knees if lying down isn't possible. Loosen tight clothing and ensure fresh air. Let them get up slowly once recovered.

> **Warning**
> Seek medical help if the person was injured in a fall, doesn't regain consciousness quickly, faints repeatedly, or the fainting is linked to chest pain or an irregular heartbeat.

8. Seizures

- **Recognise:** Sudden convulsions, stiffening, or loss of awareness, sometimes with jerking movements.
- **Respond:** Stay calm, clear the area of nearby objects, and cushion the person's head. Do not restrain their movements. Time the seizure. Once it ends, gently turn them onto their side.

Emergency

Call emergency services if it is a first-ever seizure, lasts longer than 5 minutes, another seizure follows immediately, the person is injured, pregnant, or diabetic, or they do not regain consciousness afterward.

Common Mistake

Never put anything in the person's mouth during a seizure. This outdated advice can cause serious injury — it is not possible to "swallow your tongue."

9. Diabetic Emergencies

Condition	Signs	What To Do
Low blood sugar (Hypoglycaemia)	Shakiness, sweating, confusion, irritability, hunger, pale skin	If conscious and able to swallow, give something sugary (glucose tablets, fruit juice, sugar water); recheck after 15 minutes
High blood sugar (Hyperglycaemia)	Excessive thirst, frequent urination, fatigue, fruity-smelling breath, confusion	Encourage water if conscious; this develops more slowly — seek medical help; do not give insulin yourself

Tip

If you are unsure whether blood sugar is high or low and the person is conscious, giving a small amount of sugar is generally the safer first step, since low blood sugar is the more immediately dangerous condition. Always call for medical help regardless.

10. Allergic Reactions (Anaphylaxis)

- **Recognise:** Swelling of the face, lips, tongue, or throat, difficulty breathing, widespread hives, rapid pulse, dizziness, or a feeling of impending doom.
- **Respond:** Call emergency services immediately. If the person has their own prescribed adrenaline auto-injector, help them use it as directed. Help them lie flat with legs raised, unless breathing difficulty makes sitting up more comfortable.

Emergency

Anaphylaxis can worsen within minutes. Be ready to begin CPR if the person becomes unresponsive and stops breathing normally. A second dose of their auto-injector may be needed if there's no improvement — follow the device's instructions or the emergency operator's guidance.

11. Poisoning

- **Recognise:** Nausea, vomiting, confusion, drowsiness, burns around the mouth, unusual breath odour, or difficulty breathing, often with an open container or medicine strip nearby.
- **Respond:** Call the National Poisons Information Centre (1800-116-117) or emergency services immediately. Keep the container or packaging to show responders. If the substance is on the skin or in the eyes, flush with clean water.

Common Mistake

Do not induce vomiting unless specifically instructed to by a poison control expert or medical professional — for many substances, this can cause more harm. Never give anything by mouth to an unconscious person.

12. Emergency Medical Response

How you make the emergency call matters almost as much as the first aid itself.

1. Call 112 (or 108/102 for an ambulance) as early as possible.
2. State your exact location clearly, including landmarks.
3. Describe what happened and the person's current condition.
4. Answer the dispatcher's questions calmly — they may guide you through first aid or CPR over the phone.
5. Stay on the line until told to hang up, and send someone to guide the ambulance to the exact spot.

Important Terminologies

Term	Simple Meaning
Cardiac Arrest	The heart suddenly stops pumping effectively; the person is unresponsive and not breathing normally.
CPR	Cardiopulmonary Resuscitation — chest compressions (with or without rescue breaths) used when someone isn't breathing normally.
AED	Automated External Defibrillator — a device that can restart normal heart rhythm with a controlled electric shock.
Chain of Survival	The sequence of actions — recognition, CPR, defibrillation, advanced care, recovery care — that together determine survival.
Anaphylaxis	A severe, rapid, life-threatening allergic reaction.
Hypoglycaemia	Abnormally low blood sugar.
Hyperglycaemia	Abnormally high blood sugar.
Thrombolysis	"Clot-busting" medical treatment used for certain strokes, effective only within a narrow time window.
Recovery Position	A side-lying position that keeps an unconscious but breathing person's airway clear.
Golden Hour	The critical early period after a medical emergency when treatment has the greatest effect on survival.

Step-by-Step Understanding

A General Approach to Any Medical Emergency

1. **Check for danger:** Make sure the scene is safe before approaching.
2. **Check response:** Speak loudly and gently tap the shoulders.
3. **Call for help:** Dial 112/108 immediately, or direct a specific bystander to.
4. **Check breathing:** Look, listen, and feel for normal breathing.
5. **Act on what you find:** Begin CPR, manage bleeding, support a seizure, or otherwise respond based on the signs present.
6. **Stay and monitor:** Keep checking the person's condition until professional help arrives.

Performing Hands-Only CPR, Step by Step

1. Confirm unresponsiveness and abnormal or absent breathing.
2. Position your hands on the centre of the chest, one on top of the other.
3. Keep your arms straight and use your body weight to push down firmly.
4. Compress at a steady rate of 100–120 times per minute.
5. Let the chest rise fully between each compression.
6. Continue until help arrives or the person recovers.

Real-Life Indian Examples

Example 1

At a wedding in Jaipur, a guest collapsed suddenly and stopped responding. A relative who had attended a school CPR workshop began hands-only compressions immediately while another guest called 108 — the paramedics later said the early compressions likely made the difference before the ambulance arrived.

Example 2

An office worker in Gurugram noticed a colleague's face suddenly drooping on one side and slurred speech during a meeting. Recalling the FAST test, she called for an ambulance immediately and noted the time — the colleague received clot-dissolving treatment well within the effective window.

Example 3

A school teacher in Chennai calmly guided classmates to clear space around a student having a seizure, timed it, and did not attempt to hold them down or place anything in their mouth — exactly as taught in a community health session.

Example 4

A shopkeeper in Nagpur found a customer's child had accidentally swallowed a household cleaning liquid. Instead of trying home remedies, he immediately called the National Poisons Information Centre and followed their instructions until help arrived.

Practical Scenarios

Scenario	What You Should Do
A family member clutches their chest, sweating and short of breath	Call emergency services, help them rest and stay calm, and be ready to start CPR if they become unresponsive.
A colleague suddenly can't lift one arm and their words are slurred	Run the FAST test, call emergency services immediately, and note the time symptoms began.
Someone collapses and isn't responding or breathing normally	Call for help and begin hands-only CPR immediately — don't wait to see if they recover on their own.
A known diabetic becomes shaky, confused, and sweaty	If they're conscious and can swallow, give something sugary and recheck in 15 minutes.
Someone's face and lips swell rapidly after a bee	Call emergency services and help them use their own adrenaline auto-injector if they

sting	carry one.

Government Rules & Guidelines

Body / Programme	Role
National Programme for Prevention & Control of Cancer, Diabetes, CVD & Stroke (NPCDCS)	The Ministry of Health & Family Welfare's national framework for tackling major non-communicable diseases, including heart disease and stroke.
National Poisons Information Centre (NPIC), AIIMS New Delhi	India's leading 24x7 poisoning information service, established in 1995, offering toll-free guidance on poison exposure and management.
Indian Resuscitation Council (IRC)	Developed India-specific Compression-Only-Life-Support (COLS) guidelines to promote realistic, hands-only CPR training suited to Indian conditions.
ERSS-112	India's unified Emergency Response Support System connecting citizens to police, fire, and medical help through one number.

Laws & Policies

Good Samaritan Protection Applies Here Too

The same Good Samaritan Law that protects those who help road accident victims (recognised by the Supreme Court of India in 2016 and reflected in the Motor Vehicles Amendment Act, 2019) also removes the fear of civil or criminal liability for anyone who steps in, in good faith, during a medical emergency — including performing CPR on a stranger.

India does not yet have a dedicated national law mandating CPR training in schools or workplaces, unlike some countries — a gap that medical bodies including the Cardiological Society of India and Indian Resuscitation Council Federation have publicly called to close. Until then, awareness platforms like JanGyan play a genuine role in closing this gap.

Important Statistics

Statistic	Insight
Bystander CPR is given in only about 1.3%–9.8% of cardiac arrests in India	Far below the global target of over 60%, set by international resuscitation bodies.
Cardiac arrest survival in India is estimated at under 10%	Shows how much early bystander action could improve outcomes.
A stroke occurs roughly every 20 seconds in India, according to clinical reports	Highlights the scale of the challenge and the importance of FAST recognition.
Clot-dissolving stroke treatment is generally only effective within about 4.5 hours of symptom onset	Explains why calling for help immediately, and noting the time, is so critical.

Suggested Visual: A "Chain of Survival" flow diagram with icons for each link.

Best Practices

- Learn hands-only CPR through a recognised course whenever the opportunity arises.
- Save 112, 108/102, and the National Poisons Information Centre number (1800-116-117) in your phone.
- Act on FAST signs immediately — never "wait and see" with stroke or heart attack symptoms.
- Keep a list of any family member's known allergies, medications, or conditions (like diabetes) accessible.
- Stay with the person and keep monitoring them until help arrives.
- Remain calm — your calmness helps keep the person, and yourself, more in control.

Common Mistakes

- Waiting to see if chest pain or stroke symptoms "go away on their own."
- Putting an object in the mouth of someone having a seizure.
- Giving food, water, or medication by mouth to someone who is confused or unconscious.
- Inducing vomiting after a suspected poisoning without professional guidance.
- Hesitating to start CPR out of fear of doing it "wrong" or facing legal trouble.
- Restraining a person's movements during a seizure.

Myths vs Facts

Myth	Fact
"You can swallow your tongue during a seizure."	This is physically impossible — never put anything in a seizing person's mouth.
"CPR requires mouth-to-mouth breathing to work."	Hands-only CPR (compressions only) is recommended for untrained bystanders and is effective in the first minutes.
"If someone survived the initial heart attack symptoms, they're out of danger."	The risk remains high for hours afterward — emergency evaluation is still essential.
"Only older people have strokes or heart attacks."	Both can affect younger adults, especially with rising lifestyle risk factors.
"You should always try to make a poisoning victim vomit."	This is now known to cause additional harm for many substances — call poison control first.

Do's and Don'ts

Do	Don't
Call for help immediately when you notice warning signs	Don't wait to see if symptoms improve on their own
Start hands-only CPR on an unresponsive, non-breathing person	Don't hesitate out of fear of doing it imperfectly
Note the exact time symptoms started for stroke or heart	Don't give food, water, or medicine to someone confused or

attack	unconscious
Help someone use their own prescribed emergency medication	Don't offer your own medication or guess a dose
Keep the poison container for responders to see	Don't induce vomiting without professional guidance

Safety Tips

Know Your Protection

India's Good Samaritan Law protects you from civil and criminal liability when you help someone in good faith during a medical emergency, including performing CPR on a stranger.

- Protect yourself first — check for danger before approaching.
- Use a barrier or cloth if available when giving compressions to a stranger, though this should never delay starting CPR.
- Never attempt an unfamiliar medical procedure beyond what is covered in this handbook.
- Keep emergency numbers saved and visible for every member of your household.

Emergency

Dial **112** for unified emergency response, **108/102** for an ambulance, or **1800-116-117** (toll-free) for the National Poisons Information Centre.

Practical Checklist

Emergency Readiness Checklist

☐ 112, 108/102, and 1800-116-117 saved in your phone
☐ Know the FAST test for stroke by heart
☐ Comfortable with the basic steps of hands-only CPR
☐ Aware of any family member's diabetes, allergies, or heart conditions
☐ Know the location of the nearest hospital with emergency care
☐ Understand your Good Samaritan Law protections

Quick Revision Notes

Revision

- Chain of Survival: Recognition & Call → CPR → Defibrillation → Advanced Care → Recovery Care.
- FAST for stroke: Face, Arms, Speech, Time.
- Hands-only CPR: push hard and fast, 100–120 compressions per minute, don't stop.
- Seizures: never restrain, never put anything in the mouth, time it, call for help if it lasts over 5 minutes.
- Poisoning: don't induce vomiting; call 1800-116-117 or 112.
- Good Samaritan Law protects bystanders who help in good faith, including during CPR.

Key Takeaways

Key Takeaway

1. Recognising the warning signs early is often more valuable than any single treatment.
2. Hands-only CPR is simple enough for any untrained bystander to perform.
3. Calling for help immediately is always the right first move, in every scenario in this handbook.
4. Many "helpful" instincts — inducing vomiting, restraining a seizure, giving water to the unconscious — are actually harmful.
5. Indian law protects you for stepping forward in good faith.

Glossary

Cardiac Arrest — Sudden loss of effective heart pumping; the person is unresponsive and not breathing normally.

CPR — Cardiopulmonary Resuscitation, chest compressions used to maintain blood flow.

AED — Automated External Defibrillator, a device that can restart a normal heart rhythm.

FAST — Face, Arms, Speech, Time — the standard stroke recognition test.

Anaphylaxis — A severe, rapid, life-threatening allergic reaction.

Hypoglycaemia — Dangerously low blood sugar.

Chain of Survival — The linked sequence of actions that together determine survival in cardiac arrest.

Thrombolysis — Clot-dissolving treatment for certain strokes, effective only within a narrow time window.

Frequently Asked Questions

1. What is the difference between a heart attack and cardiac arrest?

A heart attack is a blocked blood supply to the heart muscle; cardiac arrest is when the heart stops pumping altogether and the person becomes unresponsive.

2. What does FAST stand for?

Face, Arms, Speech, Time — the standard way to recognise a possible stroke.

3. Do I need to give rescue breaths during CPR?

For untrained bystanders, hands-only (compression-only) CPR is recommended and effective.

4. How fast should chest compressions be during CPR?

About 100 to 120 compressions per minute, pushing hard on the centre of the chest.

5. Should I put something in the mouth of someone having a seizure?

No — this is a dangerous myth and can cause injury.

6. What should I do if someone's blood sugar seems very low?

If they're conscious and can swallow, give something sugary and recheck after 15 minutes.

7. What is anaphylaxis?

A severe, rapid, life-threatening allergic reaction that can affect breathing within minutes.

8. Should I try to make someone vomit after suspected poisoning?

No — call the National Poisons Information Centre or emergency services first.

9. What is the National Poisons Information Centre's number?

1800-116-117, a toll-free, 24x7 service run by AIIMS New Delhi.

10. How long is the effective window for stroke treatment?

Generally up to about 4.5 hours from when symptoms first began, which is why timing matters so much.

11. What should I do if someone faints?

Help them lie down and raise their legs, loosen tight clothing, and let them get up slowly once recovered.

12. Am I protected by law if I perform CPR on a stranger?

Yes — India's Good Samaritan Law protects those who help in good faith during emergencies.

13. What is an AED and can anyone use one?

An Automated External Defibrillator; yes, it is designed to guide even untrained users with voice instructions.

14. How can I tell if someone is in cardiac arrest?

They are unresponsive and not breathing normally, or only gasping.

15. What's the safest response if I'm not sure whether someone's diabetes emergency is high or low blood sugar?

If they're conscious, a small amount of sugar is generally the safer first step, alongside calling for medical help.

16. Should I give an unconscious person water?

No — this can cause choking. Focus on checking breathing and calling for help.

17. What are the warning signs of a heart attack in women?

Symptoms can be subtler — unusual fatigue, nausea, or back or jaw pain, sometimes without obvious chest pain.

18. How long should I continue CPR?

Until the person shows signs of life, an AED arrives, or professional help takes over.

19. Can I help someone use their own adrenaline auto-injector?

Yes — helping them use their own prescribed device as directed is appropriate; do not use someone else's device on them.

20. What number should I call in a medical emergency in India?

112 for unified emergency response, or 108/102 for an ambulance, depending on your state.

21. Is bystander CPR common in India?

No — studies estimate it happens in only about 1.3%–9.8% of cases, far below the global target.

22. What should I say when I call emergency services?

Your exact location, what happened, the person's condition, and answer the dispatcher's questions calmly.

23. Can a stroke happen to a young, healthy-looking person?

Yes — while risk rises with age, strokes can affect younger adults too.

24. What is the "Chain of Survival"?

The linked sequence — recognition and calling for help, CPR, defibrillation, advanced care, and recovery care — that together determine survival.

25. Where can I learn hands-on CPR training in India?

Through recognised courses from the Indian Red Cross Society, St John Ambulance India, and hospital-run community programmes.

Suggested Infographics, Flowcharts & Tables

Flowchart: The Chain of Survival, shown as five connected links.

Illustration: Correct hand position and posture for hands-only CPR.

Poster: The FAST stroke test, shown as four simple icons.

Infographic: "What to do vs What NOT to do" for seizures.

Table/Card: Key emergency numbers for every household to keep visible.

Official References

- Ministry of Health & Family Welfare, Government of India — National Programme for Prevention & Control of Cancer, Diabetes, CVD & Stroke (NPCDCS).
- National Poisons Information Centre (NPIC), AIIMS New Delhi — poisoning management and helpline information.
- Indian Resuscitation Council — Compression-Only-Life-Support (COLS) guidelines.
- Cardiological Society of India and Indian Heart Journal — bystander CPR and cardiac arrest outcome data.
- Supreme Court of India — Savelife Foundation & Anr. v. Union of India & Anr. (2016), Good Samaritan protections.
- Indian Red Cross Society and St John Ambulance India — standard first aid and CPR training frameworks.

Official Government Website Links

Resource	Website
Ministry of Health & Family Welfare	mohfw.gov.in
National Poisons Information Centre (AIIMS)	aiims.edu
National Health Portal of India	nhp.gov.in
Indian Red Cross Society	indianredcross.org
Emergency Response Support System	112.gov.in

Conclusion

Medical emergencies are frightening precisely because they are sudden and unexpected — but as this handbook has shown, the right response to almost every one of them can be learned, remembered, and applied by an ordinary citizen with no medical background. Recognise the signs, call for help without hesitation, and act with the simple, safe steps you've learned here.

As you move ahead in the JanGyan platform, use the Practice and Quiz sections to reinforce what you've learned, and don't hesitate to use AI Assistance whenever a doubt arises. The gap between a life lost and a life saved is often no more than a few confident, well-informed minutes — and you are now better prepared to fill that gap.

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